

INPATIENT DISCHARGE REPORT

SYNTHETIC SAMPLE — FOR TESTING A CLINICAL SUMMARIZATION SYSTEM

Patient	Priya Raman (Synthetic)
Age / Sex	58 years / Female
Record ID	TEST-CARD-5816
Admission	10 August 2026
Discharge	16 August 2026
Consultant	Dr. Kavitha Menon

Presenting Complaint

The patient presented with intermittent central chest discomfort for two days, associated with exertional breathlessness and fatigue. The discomfort was described as pressure-like and lasted approximately 10–15 minutes at a time. She denied syncope or hemoptysis.

Relevant Medical History

Known history of hypertension and hyperlipidemia. No documented previous myocardial infarction. The patient reported that she had stopped taking her cholesterol medication approximately three months before admission.

Initial Assessment

On arrival, blood pressure was 156/92 mmHg, pulse 88 beats/min, respiratory rate 18/min, temperature 36.8 °C, and oxygen saturation 97% on room air. Cardiovascular examination revealed regular rhythm without a new murmur. Lungs were clear.

Investigations

Electrocardiogram showed nonspecific ST-T wave changes without persistent ST-segment elevation. Initial high-sensitivity troponin-I was mildly elevated at 38 ng/L and increased to 71 ng/L on repeat testing. Hemoglobin was 12.7 g/dL, WBC 8,900/μL, creatinine 0.9 mg/dL, and LDL cholesterol 168 mg/dL. Chest radiograph showed no acute pulmonary abnormality. Echocardiography demonstrated preserved left ventricular systolic function with an estimated ejection fraction of 58% and no major regional wall-motion abnormality.

Hospital Course

The patient was admitted for evaluation of possible acute coronary syndrome. She was placed on cardiac monitoring and treated with antiplatelet therapy, anticoagulation during the acute evaluation, a statin, and blood-pressure management. Serial cardiac enzymes were monitored. Because symptoms and biomarkers suggested a non-ST-elevation myocardial injury, cardiology performed coronary angiography. The study demonstrated moderate narrowing in one coronary vessel without an indication for immediate stent placement. The patient remained hemodynamically stable and had no recurrent chest pain after treatment.

Final Assessment

1. Non-ST-elevation acute coronary syndrome / myocardial injury, clinically improved.
2. Moderate coronary artery disease managed medically.
3. Hypertension.
4. Hyperlipidemia.

Discharge Medications

Aspirin 81 mg once daily.
Atorvastatin 40 mg once daily at night.
Metoprolol succinate 25 mg once daily.
Amlodipine 5 mg once daily.
Sublingual nitroglycerin as instructed for recurrent anginal symptoms.

Discharge Status

At discharge, the patient was alert, ambulatory, afebrile, and free of chest pain. Blood pressure had improved to 132/78 mmHg. She was tolerating oral medications and had no new neurological or respiratory symptoms.

Follow-Up

Cardiology follow-up was advised within 1–2 weeks. The patient was advised to follow a heart-healthy diet, avoid smoking exposure, take medications consistently, and gradually resume physical activity according to the cardiology team's instructions. She was instructed to seek emergency care for persistent or severe chest pain, fainting, marked breathlessness, or other rapidly worsening symptoms.

Data Quality / Evidence Notes

The initial ECG was nondiagnostic, while serial troponin values showed a rising pattern. The angiography finding was moderate rather than severe obstruction. The report therefore distinguishes objective findings from the clinical assessment and does not claim that a major coronary occlusion was demonstrated.

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